

Public Health Emergency Operations Center

« COUSP RDC »

MVE-17 Incident Management System

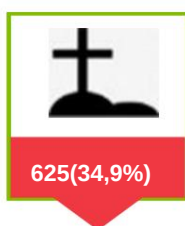
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°055/MVB_08/07/2026

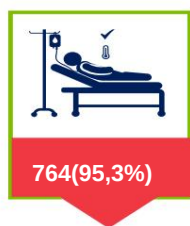
Provinces affected (3)	EAST, NORTHWEST & SOUTHWEST
Affected Health Zones (37)	- Ituri (25/36): Aru, Aungba, Bambu, Bunia, Damascus, Drodro, Gety, Kilo, Commander, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa and Boga. - North Kivu (11/34) : Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Maseka, Pottery, Balconies et Musienene. - Sud-Kivu (1/34) : Miti-Murhesa
Date the report	July 8, 2026
Publication date	July 9, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



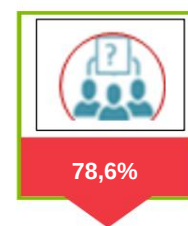
Patients in
isolation -
hospitalisation



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 2 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- Arrival of His Excellency the Minister of Public Health, Hygiene and Social Welfare, accompanied by the Secretary General for Health and some officials from the Ministry in Bunia (Ituri).
- Joint mission of the national IM and the WHO to North Kivu (Beni) to assess the response activities.
- In the past 24 hours, **33 new confirmed cases and 25 deaths (44% community transmission) have been recorded** in the provinces of Ituri (30 cases and 24 deaths) and North Kivu (3 cases and 1 death), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) outbreak in the Democratic Republic of Congo. The cumulative total now stands at **1,792** confirmed cases and **625** deaths, representing an overall case fatality rate of **34.9%**.
- Fourteen (14)** deaths were recorded among patients with Ebola virus disease in CTE/CT in Ituri (3 at CTE Bunia, 3 at CTE Rwampara, 3 at CT Nizi, 2 at CTE Mongbwalu, 1 at CTE Nyakunde, 1 at CTE CME, and 1 at CT Sota) were declared recovered after obtaining negative control tests.
- Ten (10)** patients with Ebola virus disease in Ituri province (2 at CT SOTA, 1 at CTE Rwampara, 1 at CTE Mongbwalu and 1 at CTE Nyakunde) and North Kivu (1 at HGR Oicha and HGR Mabalako) have been declared cured after obtaining negative control tests.
- Two cases have been detected in Kisangani (Tshopo), one of which is linked to the Nia-Nia health zone.** Investigations are ongoing, and the three officially affected provinces remain unchanged.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

The Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is

The population of the region is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, the Ituri province had already been affected by the 10th Ebola virus (EV) epidemic, which was then raging in North Kivu. The current epidemic, caused by Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total population estimated at nearly 15 million, with massive internal displacement and cross-border flows to neighboring countries.

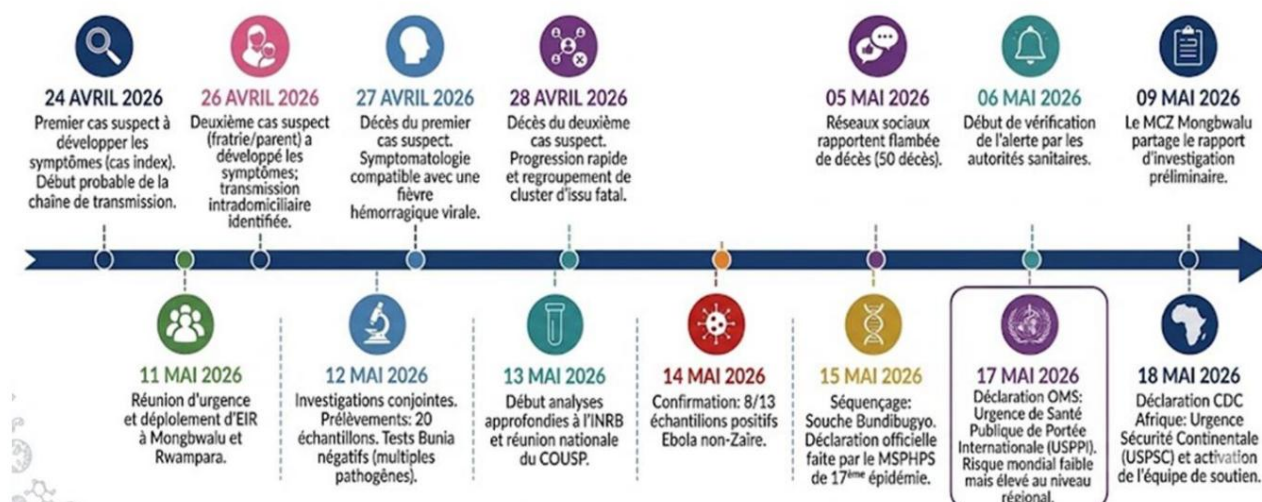


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of July 8, 2026

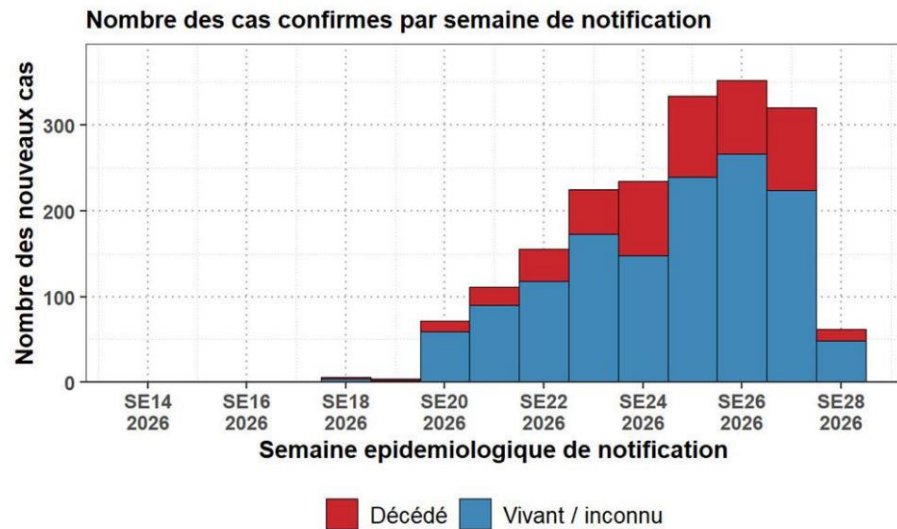
Table 1. Distribution of confirmed cases and deaths by affected province as of July 8, 2026.

Province	Confirmed cases	Death (confirmed)	Lethality	Affected health zones	New cases confirmed (24h)
Ituri	1631	535*	32,8%	25 out of 36 (69.4%) 11	30
North Kivu	158	89	56,3%	out of 34 (32.4%) 1 out	3
South Kivu	3	1	33,3%	of 34 (2.9%) 37 out	0
Total	1,792	625	34,9%	of 104 (35.6%)	33

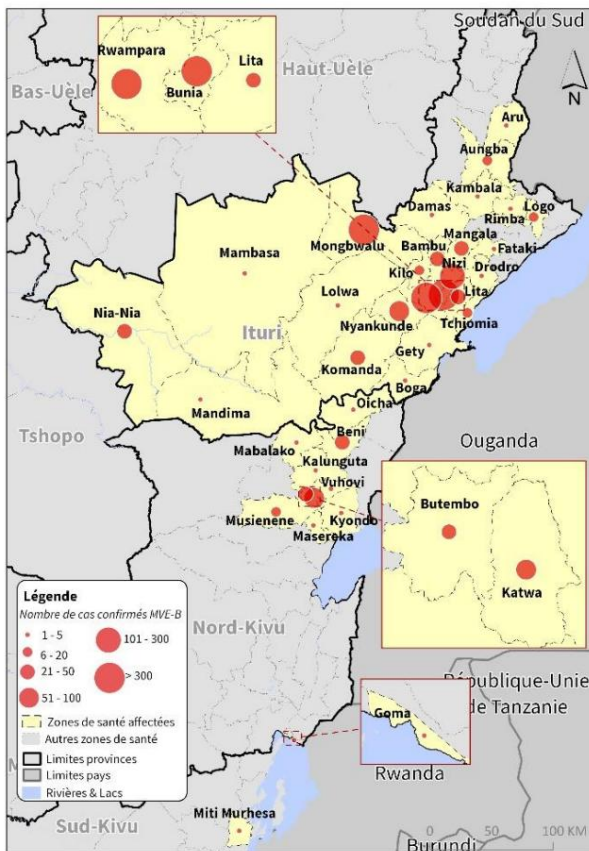
*14 deaths in Ituri were recorded among patients with EVD (previous confirmed cases) in Ebola Treatment Centers/CTs.

At the end of the day on July 8, 2026, no new health zones were affected. The number of affected health zones remains at 37/104 (section 3.1). Since May 26, 2026, South Kivu province has not recorded any confirmed cases, suggesting an absence of recent transmission.

3.2 Time-bound analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting ongoing community transmission of the disease. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capacities) confirmed continued and increased community transmission in week 25. Compared to all weeks in which cases have been recorded since the beginning of the epidemic, weeks 25 and 26 continue to show a growth in reported cases, with more than 300 cases per week.



3.3 Epidemic mapping



Figures 2 and 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of July 8, 2026.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (July 8, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR%)
ITURI	1631	535	32,8%
Bunia	487	141	29,0%
Rwampara	370	80	21,6%
Mongbwalu	315	167	53,0%
Legumes	97	22	22,7%
Nizi	104	36	34,6%
Liter	41	16	39,0%
Team	23	8	34,8%
Bamboo	29	10	34,5%
Kilo	9		11,1%
Mangala	43	1	53,5%
Tchomia	15		13,3%
Ladies	5		0,0%
Aungba	6		33,3%
Jungle	4		0,0%
Understand	3		33,3%
Washing	4		75,0%
Logo	7		0,0%
Drodoro	3		100,0%
Tide	1		0,0%
Flounder	2		100,0%
Nanny	35	23	37,1%
Fireworks	4		25,0%
Darkness	5	2 0 2 0 1 3 0 3	0 2 13 1 240,0%
Fight		1	100,0%
have		1	100,0%
Other areas not yet identified	1 1 17	0	0,0%
NORTH KIVU	158	89	56,3%
Butembo	42	20	47,6%
Cut	61	41	67,2%
Me	29	19	65,5%
Oicha	3	2	66,7%
Kalunguta	2		50,0%
Kyondo	4	1	50,0%
Ten		2	0,0%
Masereka	1	0	0,0%
Vuh			100,0%
Be careful.		0	0,0%
The museums	3	1	25,0%
SOUTH KIVU	1 1 8 3	0 2 1	33,3%
Miti-Murhesa	3	1	33,3%
TOTAL	1792	625	34,9%

Over the past 24 hours, **33 new confirmed cases and 25 deaths (44% community) have been recorded** in the provinces of Ituri (30 cases and 24 deaths) and North Kivu (3 cases and 1 death), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo. Thus, the cumulative total stands at 1,792 confirmed cases and 625 deaths, expressing an overall case fatality rate of 34.9%.

Provincial dynamics and case concentration

The **Ituri** province alone accounts for **91.0%** of cases (n=1,631) and **85.6%** of deaths (n=535), representing a crude case fatality rate of **32.8%**, making it the epicenter of the epidemic. The health zones of Bunia (487 cases), Rwampara (370 cases), Mongbwalu (315 cases), Nyankunde (97 cases), and Nizi (104) remain the most affected, accounting for approximately 84.2% of reported cases at the provincial level and 76.6% at the national level. In Nizi, the epidemic is currently progressing faster than in other areas.

Severity of the epidemic in North Kivu

A total of 158 confirmed cases and 89 deaths have been reported in North Kivu province since the start of the outbreak, representing a **crude case fatality rate of 56.3%**. This high case fatality rate is concentrated in the health zones of **Katwa** (67.2%), **Oicha** (66.7%), **Beni** (65.5%), **Kyondo** (50.0%), and **Kalunguta** (50.0%). The Katwa health zone has the highest number of reported cases (n=62), followed by the Butembo health zone (n=42) with a case fatality rate of 47.6%. A case fatality rate of 100% was observed in the exceptionally small Vuhovi health zone.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri & Kinshasa

- Arrival of His Excellency the Minister of Public Health, Hygiene and Social Welfare, accompanied by the Secretary General for Health and some officials from the Ministry in Bunia (Ituri).
- Joint mission of the national IMs and the WHO to North Kivu (Beni) to assess the response activities.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of July 8, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Report (D-1)		35	22		59
New Alerts		555	271	2	841
Total alerts for the day		590	293	15	898
Alerts investigated		319	271	17	605
Investigation rate		54,1%	92,5%	17 100,0%	67,4%
Alerts confirmed	Alive	191	16	4	211
	Deceased	55	5	0	60
Total suspected cases for today		202	21	4	271

During the day of July 8, 2026, a total of **898 alerts** were recorded, including **841** new alerts, demonstrating the continued active epidemiological surveillance in the affected provinces.

The majority were notified in Ituri **590 (65.7%)**, followed by North Kivu **271 (32.6%)** and South Kivu **15 (1.7%)**.

Of the alerts recorded that day, **898 (67.4%)** were investigated. The investigation rate reached 100% in South Kivu, while it stood at **92.5%** in North Kivu and **54.1%** in Ituri, highlighting a need to strengthen investigative capacities in this province.

The investigations led to the validation of **227** suspected cases, of which **211 (93.6%)** involved living individuals and **60 (26.4%)** community deaths. Ituri recorded the highest number of validated suspected cases with **202 (88.9%)**, followed by North Kivu with **21 (9.3%)** and South Kivu with **4 (1.8%)**.

Continued strengthening of surveillance, rapid investigation of alerts and early detection of cases, particularly in Ituri, remains essential to interrupt the chains of transmission.

Table 4. Contact tracing of confirmed cases (1) as of July 8, 2026

Province	Contacts under follow-up (n)	Contacts vus (n)	Follow-up rate (%)
Ituri	9 767	7 434	76,1%
North Kivu	2 160	1 942	89,9%
Total	11 927	9 376	78,6%

As of July 8th, **11,927** contacts were under surveillance in the provinces of Ituri and North Kivu. Of these, **9,376** were actually seen, corresponding to an overall follow-up rate of **78.6%**.

Ituri had the highest number of contacts under follow-up, 9,767 (81.9% of the total), with 7,434 contacts seen, representing a follow-up rate of 76.1%. In North Kivu, 2,160 contacts were under follow-up, of which 1,942 (89.9%) were visited.

Contact tracing performance is comparable between the two provinces; however, the overall follow-up rate remains below the recommended operational threshold for optimal contact monitoring. Strengthening daily contact tracing, particularly of those who have not been seen, remains a priority to ensure the early detection of secondary cases and limit the risk of community transmission.

Table 5. Contact tracing of confirmed cases (2) as of July 8, 2026

Province	Number of new contacts		Contact names had become suspects		Number of contacts made in the last 21 days	
	n	%	n	%	n	%
Ituri	349	70,1%	0	0,0%	61	41,2%
North Kivu	149	29,9%	3	100,0%	87	58,8%
Total	498	100,0%	3	100,0%	148	100,0%

The day ended on July 8th with **498 new contacts** listed, the majority in Ituri (349, 70.1%), while North Kivu listed 149 (29.9%). Among the contacts under monitoring, three developed symptoms consistent with the definition of a suspected case, all three in North Kivu (0.03%), requiring immediate investigation and management in accordance with current procedures.

In addition, 148 contacts successfully completed their 21-day follow-up period without developing symptoms consistent with the disease in North Kivu (87) and Ituri (61).

These results demonstrate the continuation of contact tracing and follow-up activities, while highlighting the need to maintain rigorous surveillance to ensure early detection of secondary cases and to quickly interrupt any potential chain of transmission.

Update on checkpoint and point of entry (PoC/PoE) activities

- **Ituri** : Seven (7) live alerts were notified to the PoE/PoC. The live alerts were validated, six (6) of which were isolated and referred to the CT/CTE. One alert was not transferred due to resistance from the family.

Table 6. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of July 8, 2026

INDICATORS	Ituri	North Kivu	Global
Proportion of PoC enabled and PoE enhanced	98,2% (54/55)	100,0% (14/14)	98,6% (68/69)
Number of people who went through PoE/PoC testing,	103 467	56 445	159 912
% of travelers screened, % of	93,0%	98,2%	94,9%
travelers who washed their hands, % of travelers	93,0%	98,2%	94,8%
made aware of the issue	99,5%	98,2%	99,0%
Number of alerts notified	7	0	7
Number of validated live alerts	7	0	7
Number of lifeless bodies intercepted today	0	0	0
Number of problematic contacts intercepted today; % of validated		0	0
live alerts referred to CT/CTE; % of deceased bodies swabbed	0 85,7%	ND	85,7%
	ND	ND	ND
Number of positive results of referred suspected cases	0	0	0

4.3. Laboratory

- **Ituri** : 157 samples collected and analyzed (**positivity 19.1%; n=30**).
- **North Kivu** : 113 samples collected and analyzed (**positivity 2.7%; n=3**).
- **South Kivu** : 2 samples collected and currently being analyzed.

4.4. Infection Prevention and Control (IPC)

Ituri: 19 households were identified around the cases, 18 of which were decontaminated. One identified healthcare facility was also decontaminated. Forty-two (42) deaths were recorded for the EDS activities, of which 38 EDS tests were carried out, representing a completion rate of 90%, and 37 post-mortem samples were taken.

North Kivu: Briefing of 132 health providers on the main themes of IPC took place during supervisions and support of priority structures; IPC evaluation (scorecard) was carried out in 18 health establishments in the health zones of Musienene and Beni, with an average compliance score of 25.1%, reflecting a low level of application of essential infection prevention and control measures, especially in dispensaries and private structures which are very busy with Ebola virus disease.

4.5 Holistic care

Ituri:

- **Fourteen (14)** deaths were recorded among patients with Ebola virus disease in CTE/CT in Ituri (3 at CTE Bunia, 3 at CTE Rwampara, 3 at CT Nizi, 2 at CTE Mongbwalu, 1 at CTE Nyakunde, 1 at CTE CME, and 1 at CT Sota) were declared recovered after obtaining negative control tests.
- **Ten (10)** patients with Ebola virus disease in Ituri province (2 at CT SOTA, 1 at CTE Rwampara, 1 at CTE Mongbwalu and 1 at CTE Nyakunde) and North Kivu (1 at HGR Oicha and HGR Mabalako) have been declared cured after obtaining negative control tests.
- By the end of the day on July 8th, **806** hot meals had been served, including 136 to suspected cases, 131 of the confirmed cases, 515 to PPLs and 24 to accompanying persons.

Patient movement:

- One hundred and fifty-one (151) new admissions were recorded in the three (3) provinces.

Table 7. Patient movement within healthcare facilities (CTE/CT/CI) as of July 8, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	519	193	39	751
Total admissions (24h)	103	44	4	151
Sorties (24h)				
Deceased (Suspects/Confessions)	25	0	0	25
Non-case	42	37	16	95
Healed	8	2	0	10
Escapees (Suspects/Confessions)	7	0	0	7
Transferred to the HGR	0	1	0	1
Total sorties	82	40	16	138
Patients in isolation (End of Day), including	540	197	27	764
confirmed cases (NC+AC) and	216	20	0	236
suspected cases	324	177	27	528
Overall occupancy rate	87,0%	133,0%	60%	95,3%

In total, **764** patients were in isolation, including 236 confirmed cases and 528 suspected cases, for an overall bed occupancy rate of **95.3%**.

Mental health activities and psychosocial support

Table 8. Key indicators of mental health and psychosocial support activities as of July 8, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	11 (57,9%)	4(100,0%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	63 (42,9%)	1(100,0%)	0
New suspected cases that benefited from SMSPS interventions	13 (30,2%)	2(100,0%)	0
Suspected cases under follow-up that have received interventions SMSPS	70 (52,6%)	8(100,0%)	3 (100,0%)
Laboratory results have been announced.	29	8	1
Including positive ones	9	4	1
Number of children separated in daycare who benefited from interventions SMSPS	6	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	0	7
Number of accompanying persons at the CTE who benefited from interventions SMSPS	102	6	0
Number of PPLs who benefited from SMSPS interventions	47	6	2
Number of household and community members who benefited from SMSPS interventions	39	0	0
Number of EDS supported	16	0	0

4.6 Continuity of care

- Drafting of the roadmap, the provisional budget, and the partner mapping supporting free healthcare in the Ituri province.
- Analysis of the drug stock in the health zones affected by Ebola virus disease is underway; • A total of **112 cases of pulmonary encephalopathy (PE)** have been confirmed, including **36 recoveries**, 35 deaths (31.2%), and 21 hospitalizations. the province of Ituri.

4.7 Risk communication and community engagement

Ituri: Twenty-two (22) people were engaged during a community dialogue (20 men and 2 women); 1,571 people were reached during home visits (629 men and 942 women); 3,081 people were reached during educational talks and focus groups; 3,053 people were reached during 109 mass awareness campaigns (1,170 men and 1,883 women); and 9,145 people were reached in churches, mosques and markets.

Community feedback: 7 feedback items and informational messages were collected, of which 3 were clarified, representing 42.9%. The main concerns related to free healthcare, vaccination, the availability of EDS teams, and nutritional monitoring of patients.

Support for the pillars of the response

Surveillance: 154 community alerts were reported, including 125 live alerts and 29 deaths, in the health zones of Bunia and Nizi.

CREC: 50 community health workers (29 men and 21 women) were briefed in the Nizi health zone on CREC modules and community-based surveillance. Ten (10) banners were provided to support communication efforts, and 45 journalists, including 17 women, were trained on combating health misinformation.

North Kivu: **31,007** people were reached by 1,396 Community Health Workers (CHWs) mobilized during home visits. These community health workers reached 10,545 households, comprising 18,070 women and 12,937 men, covering 98% of the daily target in the health zones of Goma, Karisimbi, Nyiragongo, and Rwanguba. The CHWs promoted preventive measures and contributed to the early detection of alerts. Capacity building and community mobilization: 63 community leaders, including 23 religious leaders and 40 community actors (CHWs, community health workers, traditional healers, teachers, etc.), had their capacities strengthened and committed to promoting Ebola prevention measures in their communities; 30 journalists received training in a dedicated briefing to improve the quality and reliability of information disseminated to the public on Ebola.

Mass communication and media: 18 local radio stations were involved in broadcasting Ebola prevention messages; 3 media formats (spots, shows and micro-programs) were broadcast on partner radio stations; 3,127 people were made aware in public places (markets, car parks, gathering points and others) of Ebola prevention measures.

Community surveillance and feedback management: 33 community alerts were reported by community actors, strengthening community surveillance and early detection of public health events; 2 rumors were documented and 100% clarified to limit the spread of misinformation; 1,835 reluctant, refusing or resistant people were convinced through community dialogues, home visits and advocacy by community leaders.

Specific feedback: In the Goma health zone, a rumor persists in Buhimba that some community members claim that Ebola virus disease (EVD) does not exist in Goma; CREC teams will reinforce the

Community dialogues are being conducted to correct this misinformation. In the Nyiragongo Health Zone, the community reported inconsistent handwashing practices at the Kanyaruchinya entry point; advocacy efforts will be undertaken with PoE/PoC teams and local officials to strengthen the systematic application of barrier measures.

4.8. Logistics

Ituri

- Reception and setup of two (2) tents for the CREC and PSEA pillars; handover of four (4) motorcycles, donated by the Government, to the Mandima and Mangala health zones; deployment of four (4) vehicles for the EDS in the Nizi health zone. In total, 25 vehicles and four (4) ambulances were made available to the various pillars to ensure their mobility.

4.9. Security

- Continued strike by service providers in the health zones of Bunia and Rwampara in Ituri.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : Community awareness-raising on the prevention of sexual exploitation and abuse continued in the health zones of Bunia and Rwampara. A total of 415 people were reached (244 women/girls and 171 men/boys), including four (4) people living with disabilities.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, strengthen team security, and increase the number of EDS teams.
2 Insufficient capacity of CTEs and saturation in North Kivu (133.0%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy additional temporary structures, and improve laboratory turnaround times.
3 Contact tracking performance not optimal (78.6% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4 Low reporting of alerts in some affected areas	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not yet ventilated by health zone (17 cases)	Major gaps in the reconstruction of transmission chains	Conduct data reconciliation, remove potential duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.). Active verification in the field.
6 Insufficient supply of MEG and medical inputs	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (112 cases) HCW (including 35 deaths, case fatality rate 31.2%)	Urgent supply, strengthening of IPC training
8 Stock of samples not yet analyzed (backlog) at the laboratory North Kivu and late diagnosis	Delayed confirmation, high lethality (56.3%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities, improve the flow of laboratory data
9 Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10 Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, and make provincial COUSPs operational.

11. Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12 Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13 Continuity of essential services (CEHS) compromise	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14. High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

6. PHOTOS OF FIELD ACTIVITIES



Briefing of the 50 RECOs in the Nizi health zone



Reception and assembly of 2 tents in the Nizi health zone

**POUR TOUTE INFORMATION
SUPPLÉMENTAIRE, VEUILLEZ CONTACTER**

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**Avec l'appui technique de l'Organisation Mondiale
de la Santé (OMS), de Africa CDC et Bluesquare**